

PMC Compass: Modelling Outputs for NER

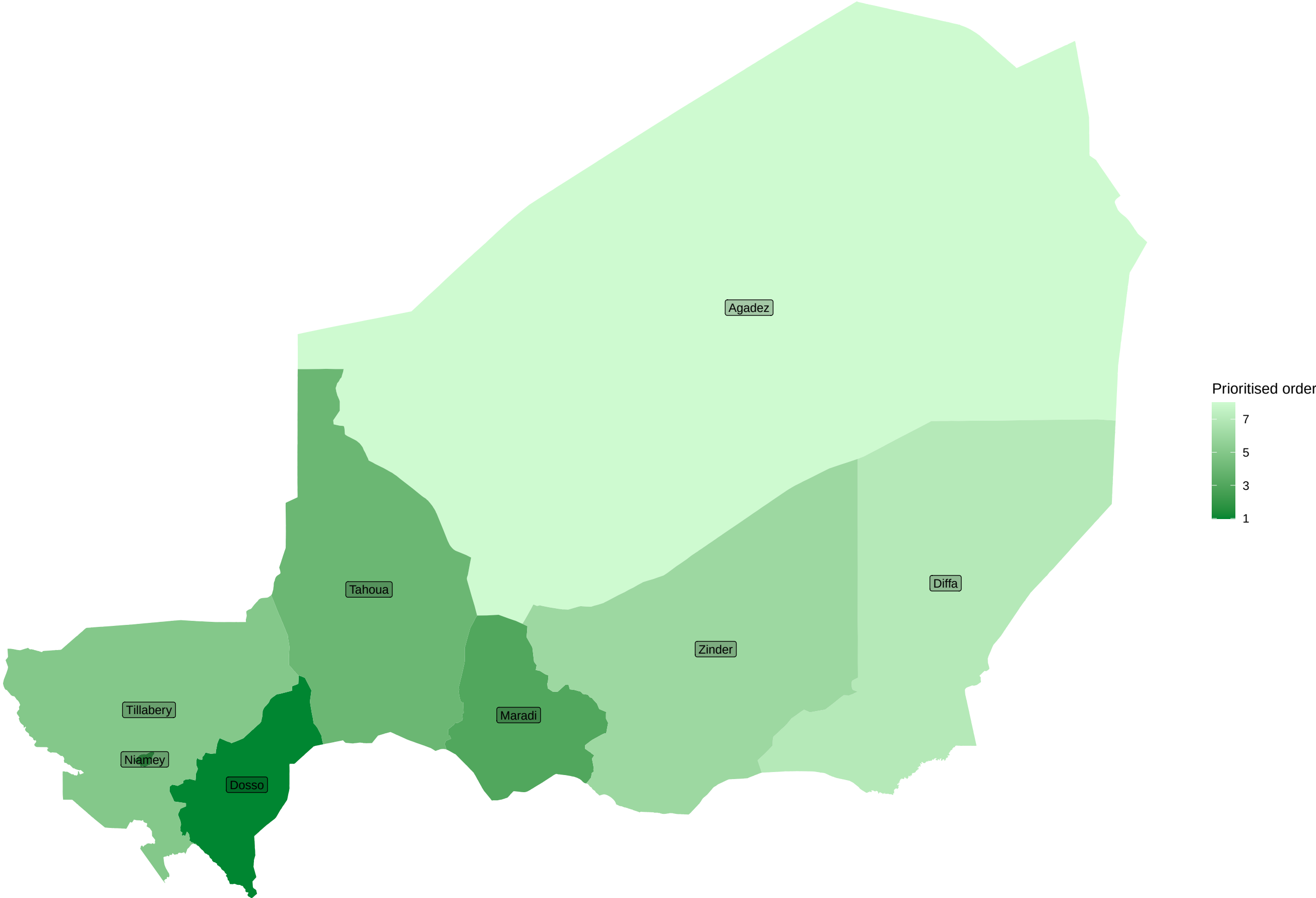
Modelling outputs from PMC Compass. Please note, results are preliminary and subject to change following further validation.

Corresponding authors: Ishana Atkinson, Catherine Pitt, Andria Mousa

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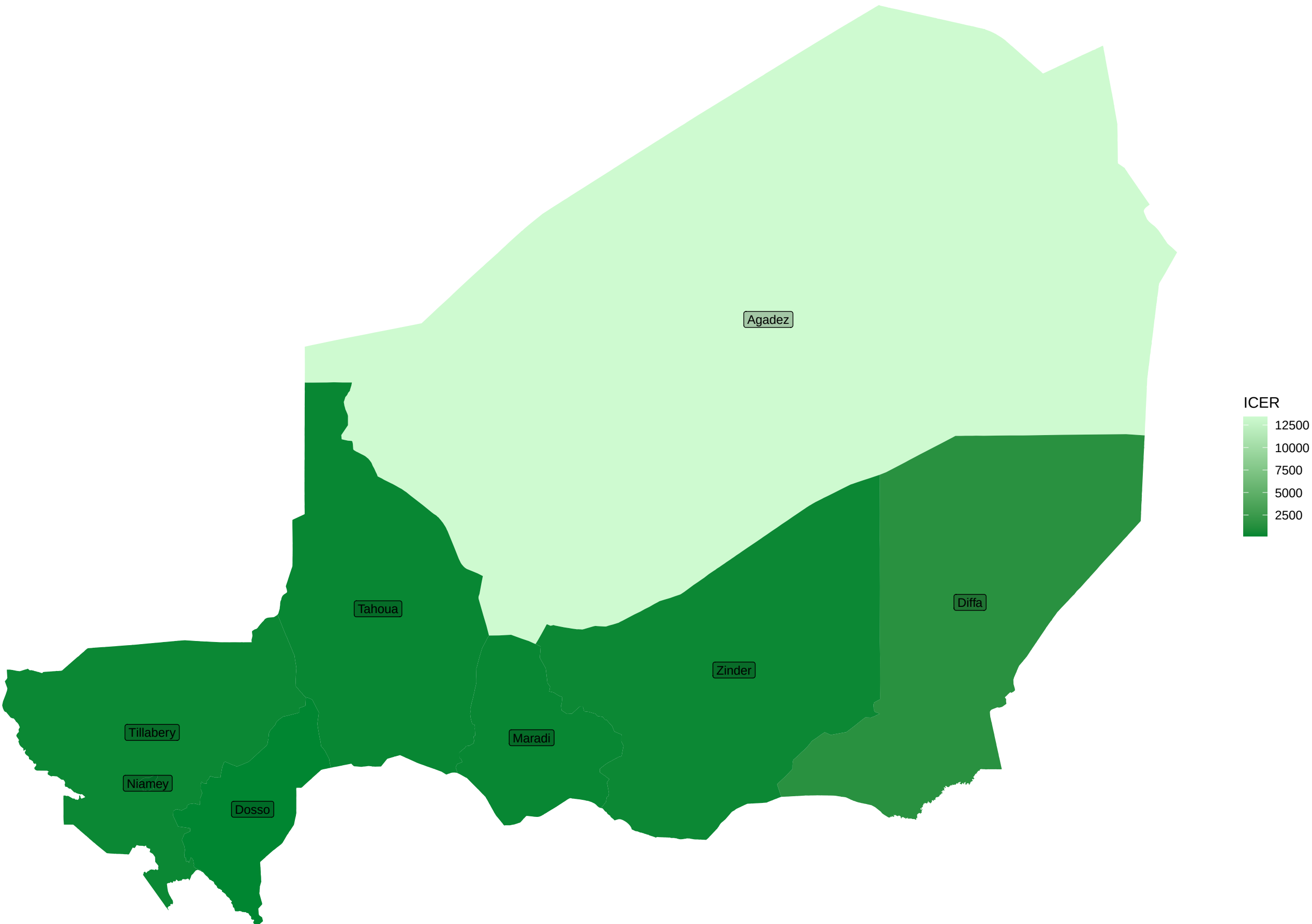
NER: Ranked prioritised order to implement PMC

Order at which to implement PMC in the country based on cost-effectiveness of the PMC delivery strategy.



NER: Incremental cost-effectiveness ratios of implementing PMC co-delivery with EPI only vs No PMC in each admin-1 unit.

Incremental cost-effectiveness ratios of implementing PMC co-delivery with EPI only vs No PMC in each admin-1 unit. All costs used to calculate these exclude national-level start up costs.



NER: Ranked prioritisation of options (page 1)

The first row indicates the single region and delivery model where PMC would be most cost-effective, meaning it would gain the most health (in disability-adjusted life-years) for the resources invested (public health service + donor costs of PMC and malaria treatment). The second row indicates the next-most cost-effective option to consider implementing alongside the first option, if additional implementation resources remain.

Country	Region (admin-1)	PMC delivery strategy (vs. comparator)	Additional clinical malaria cases averted	Additional hospitalisations averted	Additional deaths averted	Additional DALYs averted
NER	Dosso	PMC co-delivery with EPI only vs No PMC	13341	825	177	966
NER	Niamey	PMC co-delivery with EPI only vs No PMC	2693	195	42	205
NER	Maradi	PMC co-delivery with EPI only vs No PMC	10448	728	156	785
NER	Tahoua	PMC co-delivery with EPI only vs No PMC	9999	778	167	780
NER	Tillabery	PMC co-delivery with EPI only vs No PMC	6612	459	98	496
NER	Zinder	PMC co-delivery with EPI only vs No PMC	8202	555	119	611
NER	Diffa	PMC co-delivery with EPI only vs No PMC	967	68	14	73
NER	Agadez	PMC co-delivery with EPI only vs No PMC	82	9	2	8

NER: Ranked prioritisation of options (page 2)

The first row indicates the single region and delivery model where PMC would be most cost-effective, meaning it would gain the most health (in disability-adjusted life-years) for the resources invested (public health service + donor costs of PMC and malaria treatment). The second row indicates the next-most cost-effective option to consider implementing alongside the first option, if additional implementation resources remain.

Country	Region (admin-1)	PMC delivery strategy (vs. comparator)	Financial cost of SP and administration consumables	Cumulative cost of SP and administration consumables	Additional economic cost savings to public providers from reduced treatment	Cumulative economic cost savings to public providers from reduced treatment
NER	Dosso	PMC co-delivery with EPI only vs No PMC	77903	77903	-66652	-66652
NER	Niamey	PMC co-delivery with EPI only vs No PMC	35814	113717	-15033	-81685
NER	Maradi	PMC co-delivery with EPI only vs No PMC	143826	257543	-56738	-138423
NER	Tahoua	PMC co-delivery with EPI only vs No PMC	134524	392067	-58802	-197225
NER	Tillabery	PMC co-delivery with EPI only vs No PMC	107398	499465	-35811	-233036
NER	Zinder	PMC co-delivery with EPI only vs No PMC	147063	646528	-43626	-276662
NER	Diffa	PMC co-delivery with EPI only vs No PMC	23392	669920	-5287	-281949
NER	Agadez	PMC co-delivery with EPI only vs No PMC	14831	684751	-625	-282574

NER: Ranked prioritisation of options (page 3)

The first row indicates the single region and delivery model where PMC would be most cost-effective, meaning it would gain the most health (in disability-adjusted life-years) for the resources invested (public health service + donor costs of PMC and malaria treatment). The second row indicates the next-most cost-effective option to consider implementing alongside the first option, if additional implementation resources remain.

Country	Region (admin-1)	PMC delivery strategy (vs. comparator)	Additional net economic costs of PMC implementation and treatment	Cumulative net costs of PMC implementation and treatment	Incremental economic cost per DALY averted vs. next higher option in ranking	Cost-effective? (lower country-specific threshold)
NER	Dosso	PMC co-delivery with EPI only vs No PMC	325587	325587	337	no
NER	Niamey	PMC co-delivery with EPI only vs No PMC	40016	365603	195	no
NER	Maradi	PMC co-delivery with EPI only vs No PMC	276273	641876	352	no
NER	Tahoua	PMC co-delivery with EPI only vs No PMC	280032	921908	359	no
NER	Tillabery	PMC co-delivery with EPI only vs No PMC	206548	1128456	416	no
NER	Zinder	PMC co-delivery with EPI only vs No PMC	259933	1388389	425	no
NER	Diffa	PMC co-delivery with EPI only vs No PMC	109165	1497554	1495	no
NER	Agadez	PMC co-delivery with EPI only vs No PMC	107107	1604661	13388	no

NER: Ranked prioritisation of options (page 4)

The first row indicates the single region and delivery model where PMC would be most cost-effective, meaning it would gain the most health (in disability-adjusted life-years) for the resources invested (public health service + donor costs of PMC and malaria treatment). The second row indicates the next-most cost-effective option to consider implementing alongside the first option, if additional implementation resources remain.

Country	Region (admin-1)	PMC delivery strategy (vs. comparator)	Cost-effective? (higher country-specific threshold)	Cost-effective? (\$250 / DALY averted threshold)	Population (all ages)	Expected PMC doses delivered (age 0-1yr)
NER	Dosso	PMC co-delivery with EPI only vs No PMC	no	no	2964243	486891
NER	Niamey	PMC co-delivery with EPI only vs No PMC	yes	yes	1329334	223836
NER	Maradi	PMC co-delivery with EPI only vs No PMC	no	no	5341429	898911
NER	Tahoua	PMC co-delivery with EPI only vs No PMC	no	no	5769301	840774
NER	Tillabery	PMC co-delivery with EPI only vs No PMC	no	no	4447279	671235
NER	Zinder	PMC co-delivery with EPI only vs No PMC	no	no	6068830	919145
NER	Diffa	PMC co-delivery with EPI only vs No PMC	no	no	1016164	146201
NER	Agadez	PMC co-delivery with EPI only vs No PMC	no	no	772897	92694

NER: Ranked prioritisation of options (page 5)

The first row indicates the single region and delivery model where PMC would be most cost-effective, meaning it would gain the most health (in disability-adjusted life-years) for the resources invested (public health service + donor costs of PMC and malaria treatment). The second row indicates the next-most cost-effective option to consider implementing alongside the first option, if additional implementation resources remain.

Country	Region (admin-1)	PMC delivery strategy (vs. comparator)	Expected PMC doses delivered (age 1-2yr)	Expected PMC doses delivered (age 2-2.5yr)	Data source for cost modelling	ICER based on regional costs only (no national costs)
NER	Dosso	PMC co-delivery with EPI only vs No PMC	0	0	Plus Project countries	187
NER	Niamey	PMC co-delivery with EPI only vs No PMC	0	0	Plus Project countries	195
NER	Maradi	PMC co-delivery with EPI only vs No PMC	0	0	Plus Project countries	352
NER	Tahoua	PMC co-delivery with EPI only vs No PMC	0	0	Plus Project countries	359
NER	Tillabery	PMC co-delivery with EPI only vs No PMC	0	0	Plus Project countries	416
NER	Zinder	PMC co-delivery with EPI only vs No PMC	0	0	Plus Project countries	425
NER	Diffa	PMC co-delivery with EPI only vs No PMC	0	0	Plus Project countries	1495
NER	Agadez	PMC co-delivery with EPI only vs No PMC	0	0	Plus Project countries	13388

NER: Ranked prioritisation of options (page 6)

The first row indicates the single region and delivery model where PMC would be most cost-effective, meaning it would gain the most health (in disability-adjusted life-years) for the resources invested (public health service + donor costs of PMC and malaria treatment). The second row indicates the next-most cost-effective option to consider implementing alongside the first option, if additional implementation resources remain.

Country	Region (admin-1)	PMC delivery strategy (vs. comparator)	rank
NER	Dosso	PMC co-delivery with EPI only vs No PMC	1
NER	Niamey	PMC co-delivery with EPI only vs No PMC	2
NER	Maradi	PMC co-delivery with EPI only vs No PMC	3
NER	Tahoua	PMC co-delivery with EPI only vs No PMC	4
NER	Tillabery	PMC co-delivery with EPI only vs No PMC	5
NER	Zinder	PMC co-delivery with EPI only vs No PMC	6
NER	Diffa	PMC co-delivery with EPI only vs No PMC	7
NER	Agadez	PMC co-delivery with EPI only vs No PMC	8

NER: Cost assumptions (page 1)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	N doses in PMC schedule	Data source for cost modelling	Total cost of PMC implementation (excl national costs)
NER	Agadez	772897	No PMC	0	Plus Project countries	0
NER	Agadez	772897	PMC co–delivery with EPI only	5	Plus Project countries	107732
NER	Diffa	1016164	No PMC	0	Plus Project countries	0
NER	Diffa	1016164	PMC co–delivery with EPI only	5	Plus Project countries	114452
NER	Dosso	2964243	No PMC	0	Plus Project countries	0
NER	Dosso	2964243	PMC co–delivery with EPI only	5	Plus Project countries	247115
NER	Maradi	5341429	No PMC	0	Plus Project countries	0
NER	Maradi	5341429	PMC co–delivery with EPI only	5	Plus Project countries	333011
NER	Niamey	1329334	No PMC	0	Plus Project countries	0
NER	Niamey	1329334	PMC co–delivery with EPI only	5	Plus Project countries	55049
NER	Tahoua	5769301	No PMC	0	Plus Project countries	0
NER	Tahoua	5769301	PMC co–delivery with EPI only	5	Plus Project countries	338834
NER	Tillabery	4447279	No PMC	0	Plus Project countries	0
NER	Tillabery	4447279	PMC co–delivery with EPI only	5	Plus Project countries	242359
NER	Zinder	6068830	No PMC	0	Plus Project countries	0
NER	Zinder	6068830	PMC co–delivery with EPI only	5	Plus Project countries	303559

NER: Cost assumptions (page 2)

Detailed information on the assumptions used in estimating costs, DALYs, and cost-effectiveness used in the main ranking of options.

Iso code	Region (admin-1 unit)	Population (all ages)	Scenario	Sub-total: Cost of treatment	Total cost of PMC implementation + treatment (excluding national costs)	Total cost of PMC implementation + treatment (including full national costs for every region)
NER	Agadez	772897	No PMC	9562	9562	9562
NER	Agadez	772897	PMC co-delivery with EPI only	8937	116669	261793
NER	Diffa	1016164	No PMC	81053	81053	81053
NER	Diffa	1016164	PMC co-delivery with EPI only	75766	190218	335342
NER	Dosso	2964243	No PMC	860864	860864	860864
NER	Dosso	2964243	PMC co-delivery with EPI only	794212	1041327	1186451
NER	Maradi	5341429	No PMC	694429	694429	694429
NER	Maradi	5341429	PMC co-delivery with EPI only	637691	970702	1115826
NER	Niamey	1329334	No PMC	174918	174918	174918
NER	Niamey	1329334	PMC co-delivery with EPI only	159885	214934	360058
NER	Tahoua	5769301	No PMC	822379	822379	822379
NER	Tahoua	5769301	PMC co-delivery with EPI only	763577	1102411	1247535
NER	Tillabery	4447279	No PMC	486538	486538	486538
NER	Tillabery	4447279	PMC co-delivery with EPI only	450727	693086	838210
NER	Zinder	6068830	No PMC	642887	642887	642887
NER	Zinder	6068830	PMC co-delivery with EPI only	599261	902820	1047944

NER: Cost assumptions (page 3)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	Clinical cases (N, 0–30mo)	Clinical cases for whom care sought	Hospitalisations (N, 0–30mo)
NER	Agadez	772897	No PMC	1433	716	132
NER	Agadez	772897	PMC co–delivery with EPI only	1351	676	123
NER	Diffa	1016164	No PMC	13796	6898	1072
NER	Diffa	1016164	PMC co–delivery with EPI only	12829	6414	1004
NER	Dosso	2964243	No PMC	192925	96462	10071
NER	Dosso	2964243	PMC co–delivery with EPI only	179584	89792	9246
NER	Maradi	5341429	No PMC	133883	66942	8740
NER	Maradi	5341429	PMC co–delivery with EPI only	123435	61718	8012
NER	Niamey	1329334	No PMC	33318	16659	2213
NER	Niamey	1329334	PMC co–delivery with EPI only	30625	15312	2018
NER	Tahoua	5769301	No PMC	156730	78365	10402
NER	Tahoua	5769301	PMC co–delivery with EPI only	146731	73366	9624
NER	Tillabery	4447279	No PMC	91703	45852	6183
NER	Tillabery	4447279	PMC co–delivery with EPI only	85091	42546	5724
NER	Zinder	6068830	No PMC	120852	60426	8179
NER	Zinder	6068830	PMC co–delivery with EPI only	112650	56325	7624

NER: Cost assumptions (page 4)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	Deaths (N, 0–30mo)	Total DALYs	DALYs from clinical cases
NER	Agadez	772897	No PMC	28	119	73
NER	Agadez	772897	PMC co–delivery with EPI only	26	111	69
NER	Diffa	1016164	No PMC	230	1077	704
NER	Diffa	1016164	PMC co–delivery with EPI only	216	1004	654
NER	Dosso	2964243	No PMC	2165	13343	9839
NER	Dosso	2964243	PMC co–delivery with EPI only	1988	12377	9159
NER	Maradi	5341429	No PMC	1879	9869	6828
NER	Maradi	5341429	PMC co–delivery with EPI only	1723	9084	6295
NER	Niamey	1329334	No PMC	476	2469	1699
NER	Niamey	1329334	PMC co–delivery with EPI only	434	2264	1562
NER	Tahoua	5769301	No PMC	2236	11612	7993
NER	Tahoua	5769301	PMC co–delivery with EPI only	2069	10832	7483
NER	Tillabery	4447279	No PMC	1329	6828	4677
NER	Tillabery	4447279	PMC co–delivery with EPI only	1231	6332	4340
NER	Zinder	6068830	No PMC	1758	9009	6163
NER	Zinder	6068830	PMC co–delivery with EPI only	1639	8398	5745

NER: Cost assumptions (page 5)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	DALYs from hospitalisations	DALYs from deaths	PMC cost input: Implementation cost at national level
NER	Agadez	772897	No PMC	18	28	0
NER	Agadez	772897	PMC co–delivery with EPI only	16	26	145124
NER	Diffa	1016164	No PMC	143	230	0
NER	Diffa	1016164	PMC co–delivery with EPI only	134	216	145124
NER	Dosso	2964243	No PMC	1339	2165	0
NER	Dosso	2964243	PMC co–delivery with EPI only	1230	1988	145124
NER	Maradi	5341429	No PMC	1162	1879	0
NER	Maradi	5341429	PMC co–delivery with EPI only	1066	1723	145124
NER	Niamey	1329334	No PMC	294	476	0
NER	Niamey	1329334	PMC co–delivery with EPI only	268	434	145124
NER	Tahoua	5769301	No PMC	1383	2236	0
NER	Tahoua	5769301	PMC co–delivery with EPI only	1280	2069	145124
NER	Tillabery	4447279	No PMC	822	1329	0
NER	Tillabery	4447279	PMC co–delivery with EPI only	761	1231	145124
NER	Zinder	6068830	No PMC	1088	1758	0
NER	Zinder	6068830	PMC co–delivery with EPI only	1014	1639	145124

NER: Cost assumptions (page 6)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	PMC cost input: PMC implementation cost per district	PMC cost input: PMC implementation cost per health facility	PMC cost input: Cost of PMC administration consumables per dose
NER	Agadez	772897	No PMC	0	0	0.00
NER	Agadez	772897	PMC co–delivery with EPI only	24172	139	0.05
NER	Diffa	1016164	No PMC	0	0	0.00
NER	Diffa	1016164	PMC co–delivery with EPI only	24172	139	0.05
NER	Dosso	2964243	No PMC	0	0	0.00
NER	Dosso	2964243	PMC co–delivery with EPI only	24172	139	0.05
NER	Maradi	5341429	No PMC	0	0	0.00
NER	Maradi	5341429	PMC co–delivery with EPI only	24172	139	0.05
NER	Niamey	1329334	No PMC	0	0	0.00
NER	Niamey	1329334	PMC co–delivery with EPI only	24172	139	0.05
NER	Tahoua	5769301	No PMC	0	0	0.00
NER	Tahoua	5769301	PMC co–delivery with EPI only	24172	139	0.05
NER	Tillabery	4447279	No PMC	0	0	0.00
NER	Tillabery	4447279	PMC co–delivery with EPI only	24172	139	0.05
NER	Zinder	6068830	No PMC	0	0	0.00
NER	Zinder	6068830	PMC co–delivery with EPI only	24172	139	0.05

NER: Cost assumptions (page 7)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	PMC Cost input: Cost per SP tablet (includes wastage)	N districts in region	N health facilities in region
NER	Agadez	772897	No PMC	0.00	3	180
NER	Agadez	772897	PMC co–delivery with EPI only	0.11	3	180
NER	Diffa	1016164	No PMC	0.00	3	186
NER	Diffa	1016164	PMC co–delivery with EPI only	0.11	3	186
NER	Dosso	2964243	No PMC	0.00	5	523
NER	Dosso	2964243	PMC co–delivery with EPI only	0.11	5	523
NER	Maradi	5341429	No PMC	0.00	6	641
NER	Maradi	5341429	PMC co–delivery with EPI only	0.11	6	641
NER	Niamey	1329334	No PMC	0.00	1	45
NER	Niamey	1329334	PMC co–delivery with EPI only	0.11	1	45
NER	Tahoua	5769301	No PMC	0.00	7	555
NER	Tahoua	5769301	PMC co–delivery with EPI only	0.11	7	555
NER	Tillabery	4447279	No PMC	0.00	6	169
NER	Tillabery	4447279	PMC co–delivery with EPI only	0.11	6	169
NER	Zinder	6068830	No PMC	0.00	5	587
NER	Zinder	6068830	PMC co–delivery with EPI only	0.11	5	587

NER: Cost assumptions (page 8)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	N expected PMC doses delivered (age 0–1yr)	N expected PMC doses delivered (age 1–2yr)	N expected PMC doses delivered (age 2–2.5yr)
NER	Agadez	772897	No PMC	0	0	0
NER	Agadez	772897	PMC co–delivery with EPI only	92694	0	0
NER	Diffa	1016164	No PMC	0	0	0
NER	Diffa	1016164	PMC co–delivery with EPI only	146201	0	0
NER	Dosso	2964243	No PMC	0	0	0
NER	Dosso	2964243	PMC co–delivery with EPI only	486891	0	0
NER	Maradi	5341429	No PMC	0	0	0
NER	Maradi	5341429	PMC co–delivery with EPI only	898911	0	0
NER	Niamey	1329334	No PMC	0	0	0
NER	Niamey	1329334	PMC co–delivery with EPI only	223836	0	0
NER	Tahoua	5769301	No PMC	0	0	0
NER	Tahoua	5769301	PMC co–delivery with EPI only	840774	0	0
NER	Tillabery	4447279	No PMC	0	0	0
NER	Tillabery	4447279	PMC co–delivery with EPI only	671235	0	0
NER	Zinder	6068830	No PMC	0	0	0
NER	Zinder	6068830	PMC co–delivery with EPI only	919145	0	0

NER: Cost assumptions (page 9)

Detailed information on the assumptions used in estimating costs, DALYs, and cost-effectiveness used in the main ranking of options.

Iso code	Region (admin-1 unit)	Population (all ages)	Scenario	N total expected PMC doses delivered (age 0-2.5yr)	N SP tablets delivered	Sub-total: Cost of PMC implementation at district and HFs
NER	Agadez	772897	No PMC	0	0	0
NER	Agadez	772897	PMC co-delivery with EPI only	92694	92694	97536
NER	Diffa	1016164	No PMC	0	0	0
NER	Diffa	1016164	PMC co-delivery with EPI only	146201	146201	98370
NER	Dosso	2964243	No PMC	0	0	0
NER	Dosso	2964243	PMC co-delivery with EPI only	486891	486891	193557
NER	Maradi	5341429	No PMC	0	0	0
NER	Maradi	5341429	PMC co-delivery with EPI only	898911	898911	234131
NER	Niamey	1329334	No PMC	0	0	0
NER	Niamey	1329334	PMC co-delivery with EPI only	223836	223836	30427
NER	Tahoua	5769301	No PMC	0	0	0
NER	Tahoua	5769301	PMC co-delivery with EPI only	840774	840774	246349
NER	Tillabery	4447279	No PMC	0	0	0
NER	Tillabery	4447279	PMC co-delivery with EPI only	671235	671235	168523
NER	Zinder	6068830	No PMC	0	0	0
NER	Zinder	6068830	PMC co-delivery with EPI only	919145	919145	202453

NER: Cost assumptions (page 10)

Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	Sub–total: Cost of PMC administration (SP+consumables)	Treatment cost per clinical case	Treatment cost per hospitalisation
NER	Agadez	772897	No PMC	0	3.14	45.6
NER	Agadez	772897	PMC co–delivery with EPI only	14831	3.14	45.6
NER	Diffa	1016164	No PMC	0	3.14	45.6
NER	Diffa	1016164	PMC co–delivery with EPI only	23392	3.14	45.6
NER	Dosso	2964243	No PMC	0	3.14	45.6
NER	Dosso	2964243	PMC co–delivery with EPI only	77903	3.14	45.6
NER	Maradi	5341429	No PMC	0	3.14	45.6
NER	Maradi	5341429	PMC co–delivery with EPI only	143826	3.14	45.6
NER	Niamey	1329334	No PMC	0	3.14	45.6
NER	Niamey	1329334	PMC co–delivery with EPI only	35814	3.14	45.6
NER	Tahoua	5769301	No PMC	0	3.14	45.6
NER	Tahoua	5769301	PMC co–delivery with EPI only	134524	3.14	45.6
NER	Tillabery	4447279	No PMC	0	3.14	45.6
NER	Tillabery	4447279	PMC co–delivery with EPI only	107398	3.14	45.6
NER	Zinder	6068830	No PMC	0	3.14	45.6
NER	Zinder	6068830	PMC co–delivery with EPI only	147063	3.14	45.6

NER: Cost assumptions (page 11)

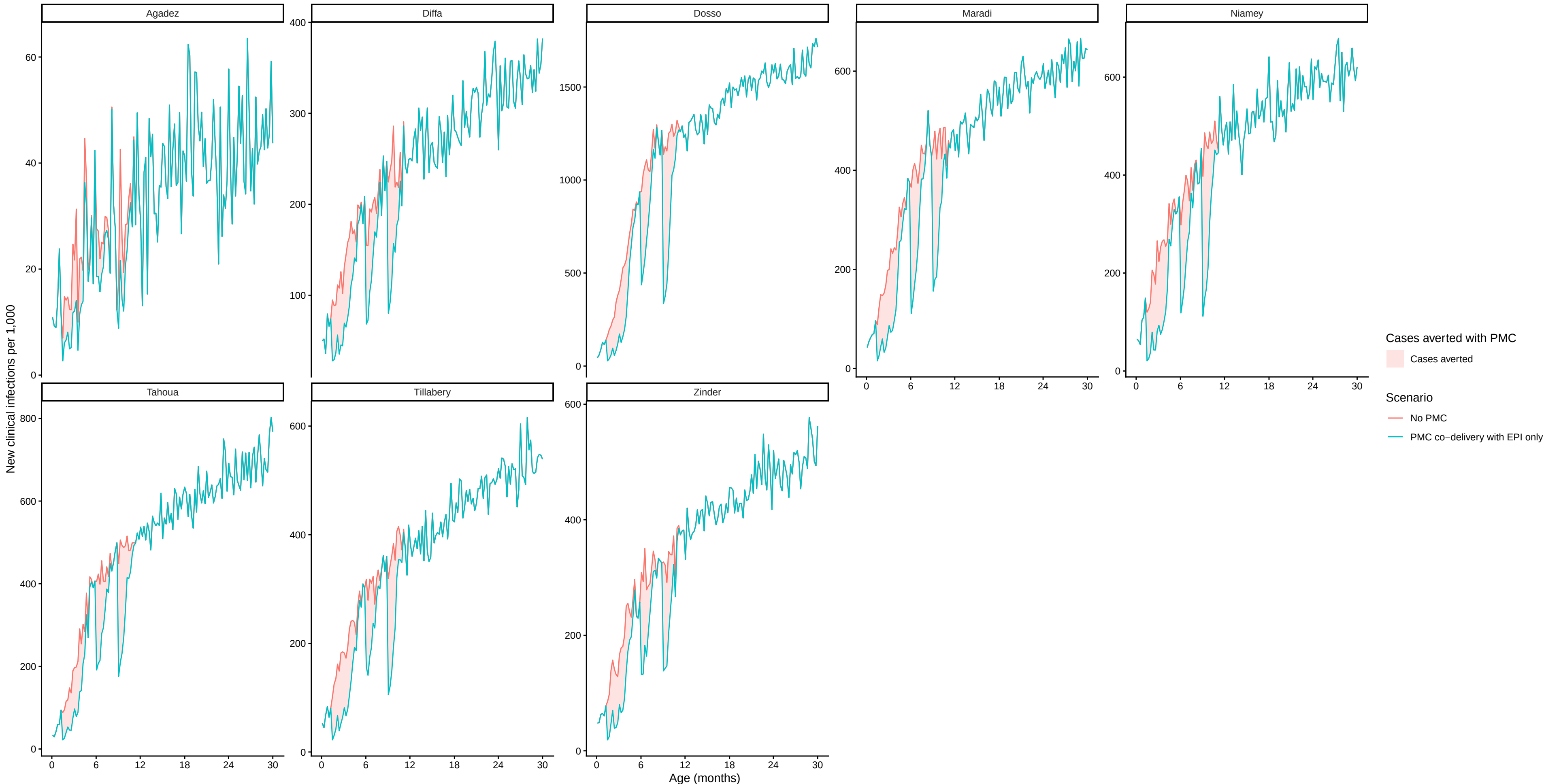
Detailed information on the assumptions used in estimating costs, DALYs, and cost–effectiveness used in the main ranking of options.

Iso code	Region (admin–1 unit)	Population (all ages)	Scenario	Treatment cost per death
NER	Agadez	772897	No PMC	45.6
NER	Agadez	772897	PMC co–delivery with EPI only	45.6
NER	Diffa	1016164	No PMC	45.6
NER	Diffa	1016164	PMC co–delivery with EPI only	45.6
NER	Dosso	2964243	No PMC	45.6
NER	Dosso	2964243	PMC co–delivery with EPI only	45.6
NER	Maradi	5341429	No PMC	45.6
NER	Maradi	5341429	PMC co–delivery with EPI only	45.6
NER	Niamey	1329334	No PMC	45.6
NER	Niamey	1329334	PMC co–delivery with EPI only	45.6
NER	Tahoua	5769301	No PMC	45.6
NER	Tahoua	5769301	PMC co–delivery with EPI only	45.6
NER	Tillabery	4447279	No PMC	45.6
NER	Tillabery	4447279	PMC co–delivery with EPI only	45.6
NER	Zinder	6068830	No PMC	45.6
NER	Zinder	6068830	PMC co–delivery with EPI only	45.6

NER: Clinical malaria incidence

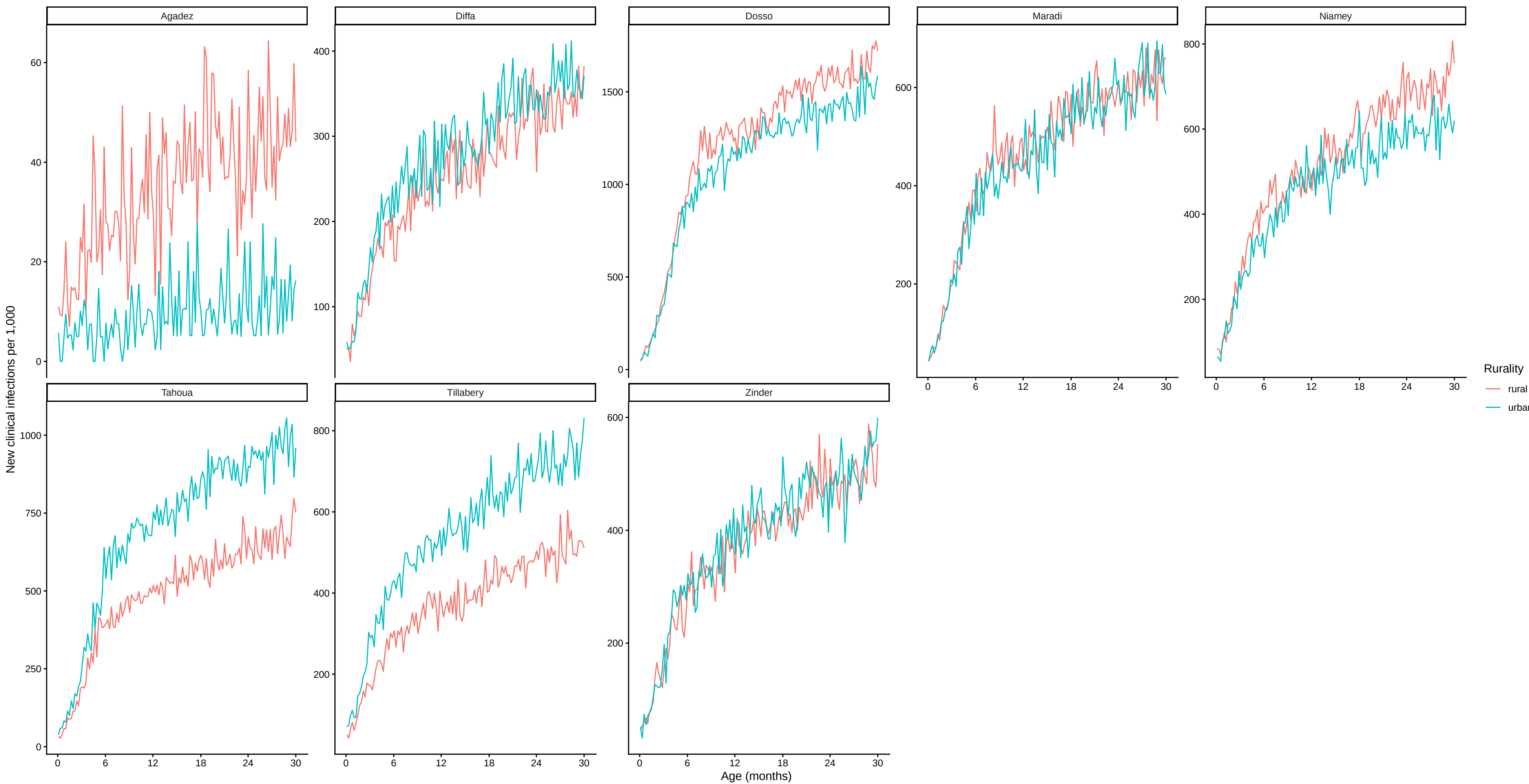
Age-specific clinical malaria cases per 1,000 children (0–30 months, with and without PMC).

PMC is assumed to be co-delivered with EPI and have a 90% uptake relative to the coverage of the co-delivered intervention.



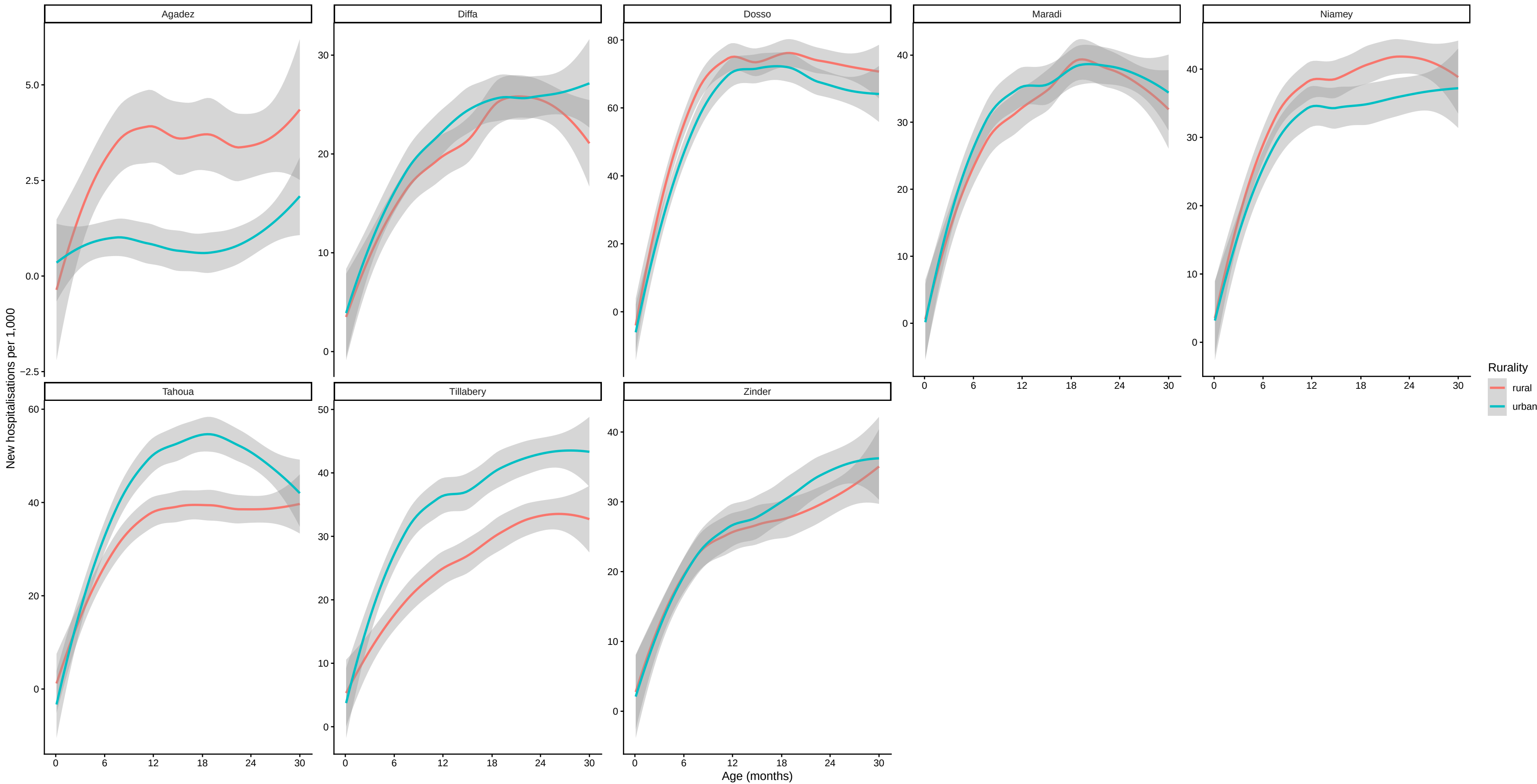
NER: Clinical malaria incidence (rural and urban)

Age-specific clinical malaria cases per 1,000 children (0–30 months, no PMC), stratified by rural and urban residence.



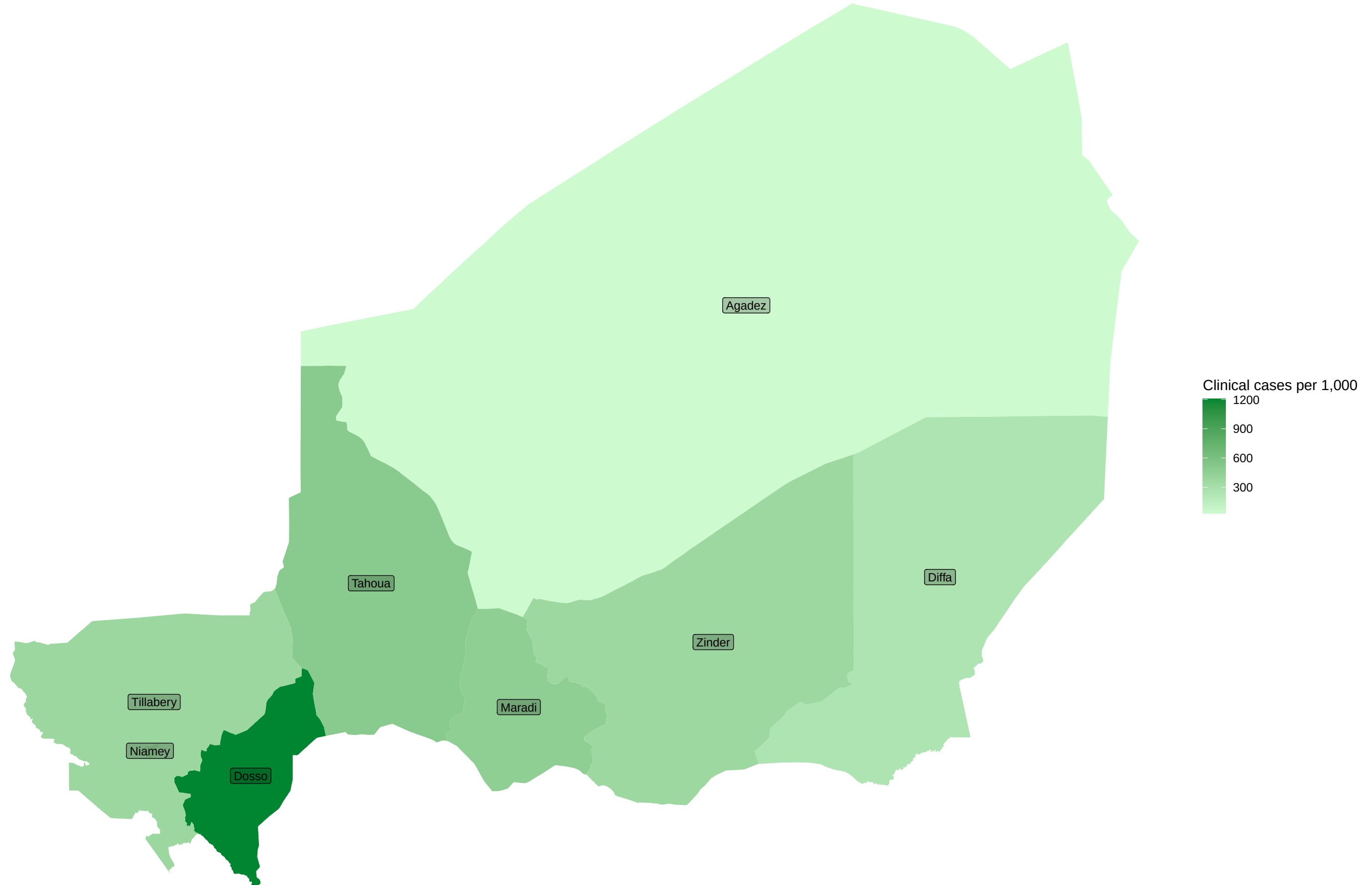
NER: Malaria hospitalisations

Age-specific new malaria hospitalisations per 1,000 children (0–30 months, no PMC), stratified by rural and urban residence.
Raw model outputs have been smoothed to reduce stochastic variation.



NER: Spatial distribution of clinical malaria without PMC

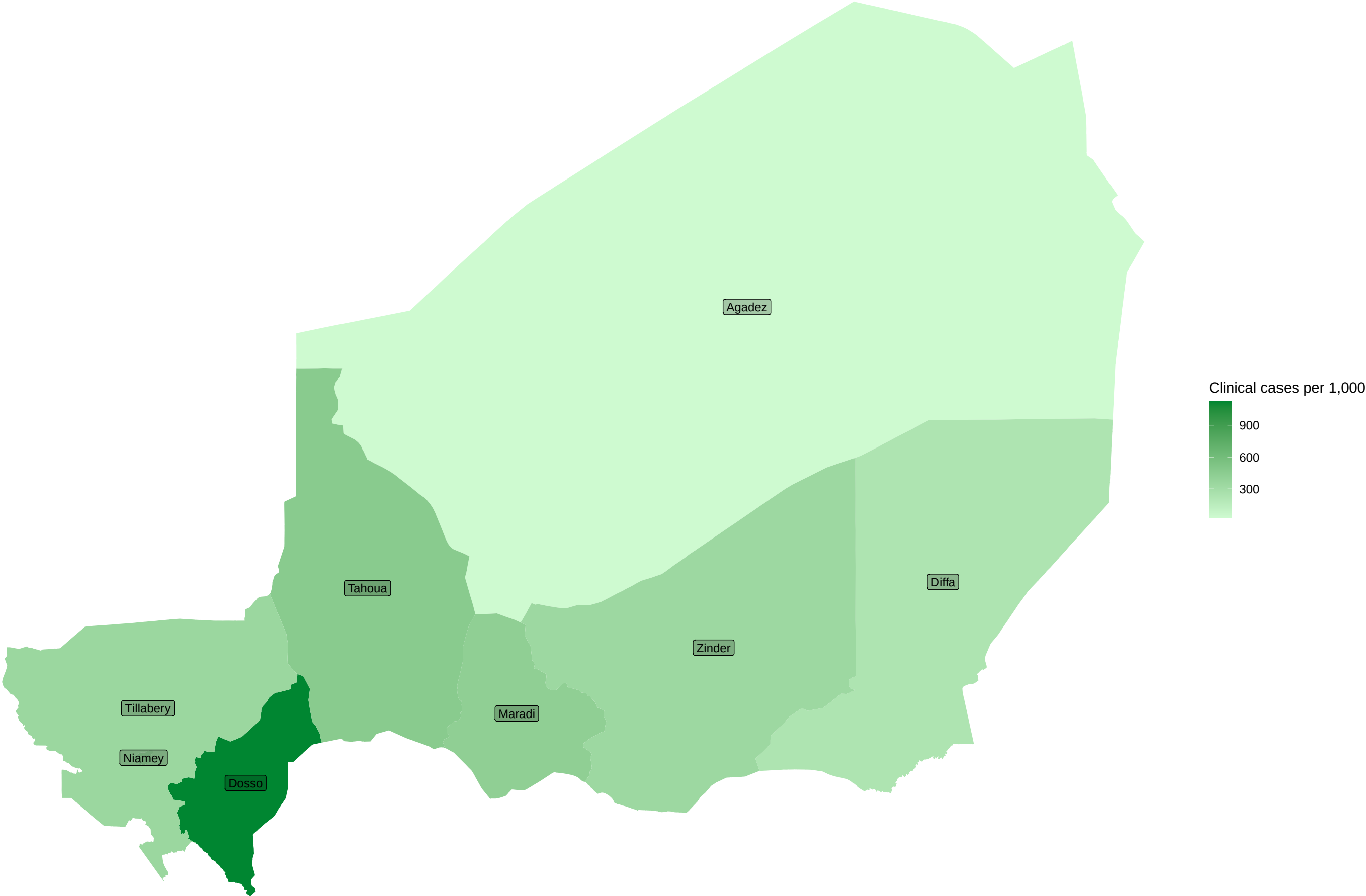
Admin-1 clinical malaria incidence per 1,000 children aged (0–30 months, no PMC).



NER: Spatial distribution of clinical malaria with PMC

Admin-1 clinical malaria incidence per 1,000 children aged (0–30 months, with PMC).

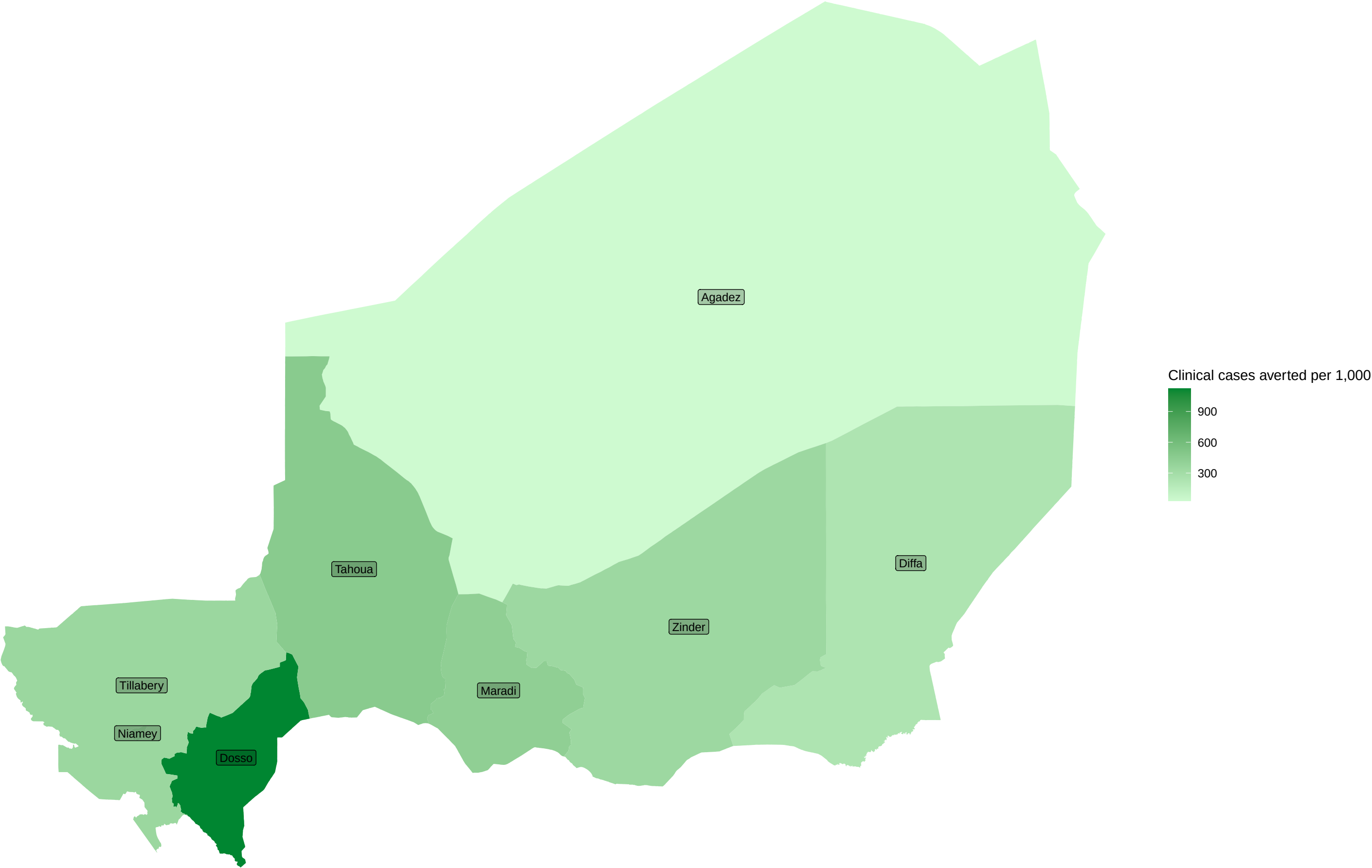
PMC is assumed to be co-delivered with EPI and have a 90% uptake relative to the coverage of the co-delivered intervention.



NER: Spatial distribution of averted clinical malaria with PMC

Admin-1 averted clinical malaria incidence per 1,000 children aged (0–30 months, averted with PMC).

PMC is assumed to be co-delivered with EPI and have a 90% uptake relative to the coverage of the co-delivered intervention.



NER: Clinical malaria burden (page 1)

Annual clinical malaria burden per 1,000 children aged 0–2.5 years, with and without PMC.

PMC is assumed to be co–delivered with EPI and have a 90% uptake relative to the coverage of the co–delivered intervention.

Admin–1	Clinical cases per 1,000, no PMC (0–2.5y)	Clinical cases per 1,000, with PMC (0–2.5y)	Clinical cases averted per 1,000 (0–2.5y)
Abia	362.1	325.8	36.3
Adamawa	379.8	355.7	24.1
Akwa_Ibom	445.1	413.0	32.1
Anambra	247.1	225.2	21.9
Bauchi	474.3	457.9	16.4
Bayelsa	578.6	539.8	38.8
Benue	369.2	343.5	25.8
Borno	1287.6	1233.4	54.3
Cross_River	527.1	486.7	40.4
Delta	333.0	308.1	24.8
Ebonyi	752.8	694.8	58.1
Edo	705.8	650.6	55.2
Ekiti	1165.1	1065.5	99.7
Enugu	380.5	348.1	32.4
Federal_Capital_Territory	323.3	297.5	25.7
Gombe	448.2	431.8	16.4
Imo	260.2	239.5	20.7
Jigawa	1614.4	1517.4	96.9
Kaduna	594.6	570.2	24.4
Kano	826.2	788.8	37.3
Katsina	2178.7	2115.4	63.2
Kebbi	3879.9	3744.2	135.7
Kogi	606.9	568.5	38.5
Kwara	372.3	348.6	23.7
Lagos	123.6	111.7	11.9
Nasarawa	1022.6	952.2	70.4
Niger	1011.6	970.3	41.3
Ogun	412.8	389.4	23.4
Ondo	1074.9	992.2	82.7
Osun	718.5	679.2	39.3

NER: Clinical malaria burden (page 2)

Annual clinical malaria burden per 1,000 children aged 0–2.5 years, with and without PMC.

PMC is assumed to be co–delivered with EPI and have a 90% uptake relative to the coverage of the co–delivered intervention.

Admin–1	Clinical cases per 1,000, no PMC (0–2.5y)	Clinical cases per 1,000, with PMC (0–2.5y)	Clinical cases averted per 1,000 (0–2.5y)
Oyo	582.4	552.3	30.1
Plateau	786.5	733.0	53.5
Rivers	384.2	352.8	31.4
Sokoto	3696.7	3601.7	95.0
Taraba	563.2	540.4	22.8
Yobe	780.6	737.7	42.9
Zamfara	3789.7	3718.0	71.7

NER: Severe malaria burden (page 1)

Annual severe malaria hospitalisations per 1,000 children aged 0–2.5 years, with and without PMC.

PMC is assumed to be co-delivered with EPI and have a 90% uptake relative to the coverage of the co-delivered intervention.

Admin-1	Hospitalisations per 1,000, no PMC (0–2.5y)	Hospitalisations per 1,000, with PMC (0–2.5y)	Hospitalisations averted per 1,000 (0–2.5y)
Abia	24.1	21.3	2.8
Adamawa	24.7	23.0	1.8
Akwa_Ibom	26.5	24.3	2.2
Anambra	16.9	15.3	1.7
Bauchi	30.9	29.7	1.2
Bayelsa	32.2	29.7	2.5
Benue	23.2	21.5	1.7
Borno	59.7	56.8	2.9
Cross_River	29.5	26.8	2.7
Delta	21.8	19.9	1.8
Ebonyi	38.3	34.9	3.4
Edo	36.8	33.1	3.7
Ekiti	48.1	42.8	5.4
Enugu	24.7	22.4	2.3
Federal_Capital_Territory	20.4	18.8	1.7
Gombe	27.8	26.6	1.2
Imo	18.4	16.8	1.6
Jigawa	72.6	67.3	5.3
Kaduna	33.2	31.6	1.6
Kano	46.5	44.1	2.3
Katsina	84.0	81.1	2.9
Kebbi	100.5	95.2	5.4
Kogi	33.3	30.6	2.7
Kwara	22.0	20.4	1.6
Lagos	10.4	9.3	1.1
Nasarawa	43.4	39.6	3.8
Niger	43.1	40.7	2.4
Ogun	25.7	23.8	1.9
Ondo	45.6	41.0	4.6
Osun	35.3	32.8	2.5

NER: Severe malaria burden (page 2)

Annual severe malaria hospitalisations per 1,000 children aged 0–2.5 years, with and without PMC.

PMC is assumed to be co–delivered with EPI and have a 90% uptake relative to the coverage of the co–delivered intervention.

Admin–1	Hospitalisations per 1,000, no PMC (0–2.5y)	Hospitalisations per 1,000, with PMC (0–2.5y)	Hospitalisations averted per 1,000 (0–2.5y)
Oyo	30.9	28.7	2.2
Plateau	39.2	35.9	3.2
Rivers	25.4	22.7	2.6
Sokoto	103.4	99.6	3.8
Taraba	32.3	30.7	1.6
Yobe	46.1	43.2	2.9
Zamfara	102.3	99.4	2.9